

**APALIFE 10 mg Tablet****APALIFE 15 mg Tablet****ANTIPSYCHOTIC**

Each film coated tablet contains:

**Aripiprazole 10 mg****Aripiprazole 15 mg**❖ **PRODUCT DESCRIPTION:****10 mg:**

Pink, modified rectangle, biconvex, film coated tablet with engraved 10 on one side and plain on the other

**15 mg:**

Yellow, round, biconvex, film coated tablet with engraved 15 on one side and plain on the other

❖ **MECHANISM OF ACTION:****Pharmacology**

It has been proposed that Aripiprazole's efficacy in schizophrenia and Bipolar I Disorder is mediated through a combination of partial agonism at dopamine D<sub>2</sub> and serotonin 5HT<sub>1a</sub> receptors and antagonism of serotonin 5HT<sub>2a</sub> receptors. Aripiprazole exhibited antagonist properties in animal models of dopaminergic hyperactivity and agonist properties in animal models of dopaminergic hypoactivity. Aripiprazole exhibited high binding affinity *in vitro* for dopamine D<sub>2</sub> and D<sub>3</sub>, serotonin 5HT<sub>1a</sub> and 5HT<sub>2a</sub> receptors and moderate affinity for dopamine D<sub>4</sub>, serotonin 5HT<sub>2c</sub> and 5HT<sub>7</sub>, alpha-1 adrenergic and histamine H<sub>1</sub> receptors. Aripiprazole also exhibited moderate binding affinity for the serotonin reuptake site and no appreciable affinity for muscarinic receptors. Interaction with receptors other than dopamine and serotonin subtypes may explain some of the other clinical effects of Aripiprazole.

**Pharmacokinetics****Absorption**

Aripiprazole is well absorbed, with peak plasma concentrations occurring within 3-5 hours after dosing. Aripiprazole undergoes minimal pre-systemic metabolism. The absolute oral bioavailability of the tablet formulation is 87%. There is no effect of a high fat meal on the pharmacokinetics of Aripiprazole.

**Distribution**

Aripiprazole is widely distributed throughout the body with an apparent volume of distribution of 4.9 l/kg, indicating extensive extravascular distribution. At therapeutic concentrations, Aripiprazole and dehydro-Aripiprazole are greater than 99% bound to serum proteins, binding primarily to albumin.

**Biotransformation**

Aripiprazole is extensively metabolized by the liver primarily by three biotransformation pathways: dehydrogenation, hydroxylation, and N-dealkylation. Based on *in vitro* studies, CYP3A4 and CYP2D6 enzymes are responsible for dehydrogenation and hydroxylation of Aripiprazole, and N-dealkylation is catalyzed by CYP3A4. Aripiprazole is the predominant medicinal product moiety in systemic circulation. At steady state, dehydro-Aripiprazole, the active metabolite, represents about 40% of Aripiprazole AUC in plasma.

**Elimination**

The mean elimination half-lives for Aripiprazole are approximately 75 hours in extensive metabolizers of CYP2D6 and approximately 146 hours in poor metabolizers of CYP2D6. The total body clearance of Aripiprazole is 0.7 mL/min/kg, which is primarily hepatic. Following a single oral dose of [<sup>14</sup>C]-labelled Aripiprazole, approximately 27% of the administered radioactivity was recovered in the urine and approximately 60% in the feces. Less than 1% of unchanged Aripiprazole was excreted in the urine and approximately 18% was recovered unchanged in the feces.

❖ **INDICATION:**

Aripiprazole is indicated for the treatment of acute episodes of schizophrenia and for maintenance of clinical improvement during continuation therapy. Aripiprazole is indicated for the treatment of acute manic episodes associated with Bipolar I Disorder.

❖ **DOSAGE AND ADMINISTRATION:****Oral****Adults**

**Schizophrenia** - The recommended starting dose for Aripiprazole is 10 or 15 mg/day administered on a once-a-day schedule without regard to meals. The maintenance dose for Aripiprazole is 15 mg/day. Doses in the range of 10 to 30 mg/day have been established as effective in clinical trials.

**Bipolar Mania** - Aripiprazole should be administered on a once-a-day schedule without regard to meals, generally starting at a dose of 15 or 30 mg/day. Dose adjustments, if indicated, should occur at intervals of not less than 24 hours. Antimanic efficacy (3-12 weeks) was demonstrated in a dose range of 15 to 30 mg/day in clinical trials. The safety of doses above 30 mg/day has not been evaluated in clinical trials.

**Special populations**

**Renal impairment** - No dosage adjustment is required in patients with renal impairment.

**Hepatic impairment** - No dosage adjustment is required for patients with hepatic impairment (Child-Pugh Class A, B, or C).

**Pediatric** - The safety and efficacy of Aripiprazole in patients under 18 years of age have not been established.

**Elderly** - No dosage adjustment is required for patients 65 years of age. However, experience with this patient population is limited.

**Gender** - No dosage adjustment is required for female patients relative to male patients.

**Patients taking medications metabolized by CYP2D6 or 3A4 – Dosage adjustment for patients taking Aripiprazole concomitantly with potent CYP3A4 or CYP2D6 inhibitors:** When concomitant administration of a potent CYP3A4 or CYP2D6 inhibitor with Aripiprazole occurs, the Aripiprazole dose should be reduced to one-half of the usual dose. When the CYP3A4 or CYP2D6 inhibitor is withdrawn from the combination therapy, the Aripiprazole dose should then be increased.

**Dosage adjustment for patients taking potent CYP3A4 inducers:** When a potent CYP3A4 inducer is added to Aripiprazole therapy, the Aripiprazole dose should be doubled. Additional dose increases of Aripiprazole should be based on clinical evaluation. When the CYP3A4 inducer is withdrawn from the combination therapy, the Aripiprazole dose should be reduced. Consideration should be given to reducing the daily dose in individual patients who are on multiple concomitant medications that inhibit CYP3A4 and CYP2D6 enzymes.

**Smoking status** - No dosage adjustment is required for smoking patients relative to non-smoking patients.

❖ **PRECAUTION:**

During antipsychotic treatment, improvement in the patient's clinical condition may take several days to some weeks. Patients should be closely monitored throughout this period.

- Suicidality, Cardiovascular disorders, Conduction abnormalities, Tardive dyskinesia, Other extrapyramidal symptoms, Neuroleptic Malignant Syndrome (NMS), Seizure, Elderly patients with dementia-related psychosis (Increased mortality, Cerebrovascular adverse reactions), Hyperglycemia and diabetes mellitus, Hypersensitivity, Weight gain, Dysphagia, Pathological gambling, Lactose, Phenylketonurics, Intolerance, Patients with ADHD comorbidity.

| Product                                                                                                                                                                                                                                                                                          | Code No.                                                 | Dimension         | Packaging Type                  | Thickness       |
|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------|-------------------|---------------------------------|-----------------|
| APALIFE                                                                                                                                                                                                                                                                                          | IAMY 0051                                                | W 16.5 x L 27 cm. | Wood Free Paper (กระดาษปลอดไม้) | 60 g (0.08 mm.) |
| <div> <div> Designed by:<br/>(PDM, ASPD, PDCM, PDCA, PDQM, PDDA) </div> <div> Checked by:<br/>(PDM/ASPD) </div> <div> Approved by:<br/>(MD, (Only Patient and Clerk)) </div> </div> <div> <div> IRA: (Date) </div> <div> QCC-PMT: (Date) </div> <div> CUSTOMER/Sales DEPT.: (Date) </div> </div> |                                                          |                   |                                 |                 |
| PM Specification                                                                                                                                                                                                                                                                                 | <div> <div> Approved by:<br/>(PLC: (Date)) </div> </div> |                   |                                 |                 |

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| Approved by:  (Date) |  | LIRA:  (Date)<br>IRA:  (Date)<br>QCC-PM:  (Date)<br>CUSTOMER/SALES DEPT.:  (Date) |           |                   |                              |                 |

## Pregnancy

### *Breast-feeding*

## Hyperglycemia and Diabetes Mellitus

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2/05/23